

1. Administrative & Study Identification

Full Study Title: A Phase II Single-arm Multi-centre Study of Volrustomig in Women With High Risk Locally Advanced Cervical Cancer Who Have Not Progressed Following Platinum Based, Concurrent Chemoradiation Therapy **Short Title/Acronym:** IVOLGA **Protocol Number:** AZ-RU-00011 **NCT Number:** NCT06943833 **Sponsor Name:** AstraZeneca **Investigational Product:** Volrustomig (MEDI5752) **Phase of Development:** Phase II **Medical Monitor:** AstraZeneca Clinical Operations Lead

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To investigate the efficacy and safety of volrustomig (MEDI5752) in women with high-risk locally advanced cervical cancer (FIGO 2018 Stage IIIA to IVA) who have not progressed following platinum-based concurrent chemoradiation therapy (CCRT). **Secondary Objectives:** To evaluate long-term survival outcomes including Overall Survival (OS) and extended Progression-Free Survival (PFS), as well as to monitor the safety and tolerability profile of volrustomig over an extended period.

2.2 Background & Rationale Patients with high-risk locally advanced cervical cancer face a significant risk of recurrence following definitive platinum-based concurrent chemoradiation therapy (CCRT). Volrustomig is a novel bispecific antibody targeting both PD-1 and CTLA-4, designed to optimize therapeutic response by maximizing CTLA-4 blockade on antigen-experienced T cells, thereby potentially increasing efficacy and minimizing toxicity. This study aims to provide clinical data on the efficacy and safety of this innovative immunotherapy for this patient population, specifically targeting the Russian Federation region.

3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional **Control Type:** Single-arm **Blinding:** Open-label **Randomization:** N/A

3.2 Planned Enrollment & Duration Target \$N\$: Approximately 36 to 85 participants **Number of Sites:** Multi-center (Russian Federation) **Estimated Study Start:** 31-Mar-2025 **Estimated Primary Completion:** 31-Mar-2028 **Estimated Study Completion:** 30-Mar-2029

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Women with histologically confirmed high-risk locally advanced cervical cancer (FIGO 2018 Stage IIIA to IVA). 2. Patients who have not progressed following platinum-based concurrent chemoradiation therapy (CCRT).

4.2 Exclusion Criteria 1. Diagnosis of small cell (neuroendocrine) or mucinous adenocarcinoma of the cervix, or evidence of metastatic disease. 2. Intent to administer a fertility-sparing treatment regimen or history of organ/allogenic stem cell transplant. 3. Patients who have undergone a previous hysterectomy or will have a hysterectomy as part of their initial cervical cancer therapy. 4. History of another primary malignancy (except for malignancy treated with curative intent with no known active disease ≥ 2 years before the first dose, or adequately treated in situ/non-melanoma skin cancer). 5. Unresolved toxicities from previous CCRT (except for irreversible toxicity not reasonably expected to be exacerbated) or active/prior documented autoimmune or inflammatory disorders.

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: Volrustomig administered as scheduled intravenous (IV) infusions. **Route of Administration:** Intravenous (IV) **Duration of Treatment:** Until Response Evaluation Criteria in Solid Tumors (RECIST) 1.1-defined radiological progression, histopathologically confirmed progression, unacceptable toxicity, withdrawal of consent, or another discontinuation criterion is met.

5.2 Concomitant & Prohibited Therapy Permitted: Standard supportive care as required. **Prohibited:** Any prior (besides prior CCRT) or concurrent treatment for cervical cancer; exposure to immune-mediated therapy prior to the study; receipt of live attenuated vaccine within 30 days prior to the first dose; and current or prior use of immunosuppressive medication within 14 days before the first dose.

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures
1	Screening	Up to Day -1 Informed Consent, Medical History, CCRT Status Evaluation, Baseline Tumor Imaging
2	Treatment	Day 1 onwards Volrustomig IV Infusion, Safety Labs, Efficacy Assessments
3	Follow-up	Up to 48 Months Ongoing Survival (OS/PFS) tracking, Adverse Event Monitoring

(Note: Specific infusion cycle cadence is determined by the study protocol.)

7. Outcome Measures (Endpoints)

7.1 Primary Endpoint Definition: Progression-free Survival at 24 months (PFS24) - The Kaplan-Meier estimate of PFS at 24 months from the date of first dose. **Measurement Method:** RECIST 1.1-defined radiological progression or histopathologically confirmed progression as assessed by the Investigator, or death due to any cause, whichever occurs earlier.

7.2 Secondary & Exploratory Endpoints 1. Overall Survival at 36 months (OS36): Kaplan-Meier estimate of OS at 36 months after study drug administration. 2. Progression-free Survival at 36 months (PFS36): Kaplan-Meier estimate of PFS at 36 months per RECIST 1.1. 3. Incidence and severity of Adverse Events (AEs).

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: AEs will be monitored from the time of ICF signature up to 48 months. **Serious Adverse Events (SAEs):** Standard reporting timelines apply (typically 24 hours of investigator awareness) to AstraZeneca Global Patient Safety. **Data Monitoring Committee (DMC):** Internal/external monitoring mechanisms established by the sponsor.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Intent-to-Treat (ITT) population for efficacy (PFS, OS); Safety Analysis Set for all patients receiving at least one dose of Volrustomig. **Sample Size Justification:** Sized to provide adequate power for Kaplan-Meier estimates of PFS at 24 and 36 months. **Handling of Missing Data:** Censoring rules applied for time-to-event endpoints (PFS, OS) per standard oncology SAP guidelines.

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki. **Monitoring Plan:** Standard clinical operations monitoring (onsite and remote) to ensure protocol adherence and data integrity. **Audit Trail:** Complete tracking of eCRF entries, data clarification processes, and source document verification.

11. Study Identifiers & Governance

Primary ID: AZ-RU-00011 **Registry Number:** NCT06943833 **Sponsor/Lead Organization:** AstraZeneca **Study Title:** IVOLGA **Official Regulatory Title:** A Phase II Single-arm Multi-centre Study of Volrustomig in

Women With High Risk Locally Advanced Cervical Cancer Who Have Not Progressed Following Platinum Based, Concurrent Chemoradiation Therapy

12. Clinical Framework (Oncology Specific)

Primary Condition: High-Risk Locally Advanced Cervical Cancer
Diagnosis Threshold: FIGO 2018 Stage IIIA to IVA **Medical Methodology:** Post-CCRT Immunotherapy Consolidation **Optimization Target:** Maintenance of disease-free status and prolongation of PFS/OS

13. Study Procedures & Methodology

Management Pathway: Screening post-CCRT, leading to initiation of Volrustomig IV infusions. **Education Protocol (HCP):** Investigator training on Volrustomig administration and immune-related AE (irAE) management. **Education Protocol (Patient):** Counseling on the investigational nature of the bispecific antibody, potential immune-related side effects, and strict adherence to the follow-up schedule. **Quality Audit Mechanism:** Independent central review of radiological imaging (if applicable) and ongoing clinical data reconciliation.

14. Observation & Follow-Up Schedule

Total Duration: Up to 48 months for safety evaluation **Onsite Visit Cadence:** Standard oncology IV infusion cycles and follow-up scans **Remote Monitoring Frequency:** As dictated by the clinical monitoring plan **Checklist Review Interval:** Continuous AE monitoring throughout the treatment and follow-up phases.

15. Sign-off & Approval

Role	Name	Signature	Date	:--	:--	:--	:--
Principal Investigator	[Name]	____	[DD-MMM-YYYY]				
Clinical Operations Lead	[Name]	____	[DD-MMM-YYYY]		Lead		
Statistician	[Name]	____	[DD-MMM-YYYY]				